PhD in Children and Adolescent Counseling

Cross-Course Review | 100 Multiple-Choice Questions

This separate review set samples all 32 listed courses: three questions per course, followed by four integrative questions. Select one best answer (A–D). Answers are withheld until the final section.

The answer key at the end includes the correct option letter, full answer text, and a brief rationale. This educational review resource does not replace faculty review, supervision, clinical judgment, or applicable law and professional standards.

CAC.901 | Advanced Developmental Psychology

001. [CAC.901] A child's behavior changes caregiver responses, and those responses then influence the child's later behavior. Which perspective best captures this reciprocal process?

A. Static trait model
B. Single-direction maturation model
C. Sensitive-period account
D. Transactional model

002. [CAC.901] A team studies how early language difficulty can affect peer relationships, which then influence later classroom participation. This is an example of:

A. A developmental cascade
B. A cross-sectional comparison
C. A fixed sensitive period
D. A cohort effect

003. [CAC.901] A counselor interprets a teenager's behavior by considering home relationships, school rules, community resources, culture, and historical context. Which framework is most directly reflected?

A. Ecological systems perspective
B. Classical conditioning only
C. Trait-only formulation
D. Single-setting assessment

CAC.902 | Theories of Counseling and Psychotherapy

004. [CAC.902] An adolescent says, "Part of me wants to stop vaping, but my friends make it hard." Which response best reflects motivational interviewing?

A. Argue that the counselor's reasons for stopping are more important.
B. Immediately prescribe a change plan without asking permission.
C. Reflect both sides of the ambivalence and invite the adolescent to describe their own reasons for change.
D. Warn the adolescent that failure to stop proves they are not motivated.

005. [CAC.902] Which combination is central to a person-centered counseling stance?

A. Rewards, response cost, and token exchange.
B. Exposure hierarchy, response prevention, and symptom monitoring.
C. Interpretation, confrontation, and therapist authority.
D. Empathic understanding, congruence, and unconditional positive regard.

006. [CAC.902] A counselor asks when a family's concern is less intense and what is different on those occasions. This question is most characteristic of:

A. Flooding-based exposure
B. Classical psychoanalysis
C. Solution-focused brief therapy
D. A purely diagnostic interview

CAC.903 | Child and Adolescent Assessment Techniques

007. [CAC.903] A measure gives very similar scores when the same stable skill is assessed under comparable conditions. This evidence primarily concerns:

A. Sensitivity to treatment
B. Ecological validity
C. Reliability
D. Clinical utility

008. [CAC.903] A screening questionnaire is above its cutoff. What is the most defensible next step?

A. Use the result to guide follow-up assessment; do not treat the screening score alone as a diagnosis.
B. Tell the family the score proves the child has a disorder.
C. Record a definitive diagnosis based only on the cutoff.
D. Disregard the result because screening tools can never be useful.

009. [CAC.903] A teacher, caregiver, and adolescent describe different behavior across school, home, and peer settings. The assessor should first:

A. Average the reports and discard the original context.
B. Integrate the reports in context and investigate why perspectives differ.
C. Assume the teacher is always the most accurate informant.
D. Select whichever report supports the first hypothesis.

CAC.904 | Ethics in Counseling Children and Adolescents

010. [CAC.904] A caregiver provides legal permission for counseling. What should the counselor also seek from a developmentally able child?

A. A waiver of all future choices about treatment.
B. A second legal consent that replaces the caregiver's authorization.
C. Assent: the child's informed, affirmative willingness to participate.
D. A promise that no information will ever be disclosed.

011. [CAC.904] Before beginning services, the counselor explains purpose, foreseeable risks, alternatives, confidentiality boundaries, and voluntary choice. This is part of:

A. A transfer summary
B. Informed consent
C. A progress note
D. A diagnostic formulation

012. [CAC.904] A disclosure may trigger a reporting duty. Which action is most appropriate?

A. Investigate independently before consulting any required procedure.
B. Promise secrecy and make no report under any circumstances.
C. Consult current jurisdictional requirements and follow applicable reporting procedures, explaining limits of privacy as appropriate.
D. Assume the same reporting rule applies in every jurisdiction.

CAC.905 | Cultural Competence in Counseling

013. [CAC.905] A counselor notices an assumption about a family's values and asks the family how they understand the issue instead of treating the assumption as fact. This best reflects:

A. Cultural humility
B. A fixed cultural checklist
C. Cultural essentialism
D. Color-blind practice

014. [CAC.905] A youth's service access is shaped jointly by disability, language, family income, and racialized barriers. Which framework best supports analysis of these interacting influences?

A. Random assignment
B. Temperament theory
C. A single-axis identity model
D. Intersectionality

015. [CAC.905] A clinician changes examples and language to fit a family's context while preserving an intervention's core active components and monitoring outcomes. This is:

A. A confidentiality exception
B. Norm-referenced scoring
C. Unstructured abandonment of the intervention
D. Cultural adaptation

CAC.906 | Trauma-Informed Care Practices

016. [CAC.906] A trauma-informed clinic explains procedures in advance, offers meaningful choices, and avoids unnecessary repeated retelling. Which principle is most evident?

A. Safety, choice, and collaboration
B. Forced disclosure
C. Confrontation as a default
D. Symptom labeling before engagement

017. [CAC.906] A young person becomes overwhelmed by a trauma reminder. The counselor first helps orient them to current sights, sounds, and the room before deeper processing. This is:

A. Grounding
B. Exposure without consent
C. Cognitive disputation
D. A diagnostic interview

018. [CAC.906] A client is too highly aroused to remain engaged in reflective discussion. Using the window-of-tolerance idea, the counselor should first:

A. Increase intensity to force immediate disclosure.
B. Continue the same task without checking the client's state.
C. Interpret the reaction as deliberate noncooperation.
D. Support regulation and safety, then resume reflective work when the client can engage.

CAC.907 | Family Systems Theory

019. [CAC.907] A child's withdrawal leads a caregiver to increase monitoring; the monitoring then increases the child's withdrawal. Which concept best describes this reciprocal sequence?

A. Cognitive restructuring
B. A cohort effect
C. Circular causality
D. Linear causation only

020. [CAC.907] Two caregivers draw their child into repeated disputes so the child carries messages between them. This interaction pattern is called:

A. A clear subsystem boundary
B. Triangulation
C. Differentiation
D. Positive reinforcement

021. [CAC.907] A therapist assesses whether caregivers can address conflict directly while keeping the child out of the mediator role. Which structural concept is most relevant?

A. Norm-referenced interpretation
B. Exposure hierarchy
C. Subsystem boundaries
D. Group universality

CAC.908 | Group Counseling for Youth

022. [CAC.908] Group members increasingly trust one another, feel they belong, and commit to the group's work. This is best described as:

A. Group cohesion
B. Extinction
C. Triangulation
D. Measurement invariance

023. [CAC.908] Before a youth group begins, the leader explains privacy expectations and clarifies that the leader cannot guarantee every member will keep information private. This is:

A. A realistic explanation of confidentiality limits in a group.
B. A guarantee that no member will ever repeat group material.
C. A promise to share every disclosure with all members.
D. A reason to omit consent and assent discussions.

024. [CAC.908] A counselor is deciding whether a youth is ready for a particular group. Which step best supports a safe, appropriate match?

A. Screen goals, readiness, safety needs, and fit with the group's purpose.
B. Enroll every referral automatically.
C. Ask current members to diagnose the prospective member.
D. Use age alone as the sole criterion.

CAC.909 | Play Therapy Techniques

025. [CAC.909] In child-centered play therapy, a child builds a tower and knocks it down. Which response best reflects tracking?

A. “Let me show you the correct way to build it.”
B. “You stacked the blocks high, and now you knocked them down.”
C. “You are angry at your father.”
D. “If you do that again, playtime is over.”

026. [CAC.909] Which statement best distinguishes directive from child-centered play therapy?

A. Directive play has no therapeutic purpose or planned goal.
B. Directive play uses therapist-selected activities for goals; child-centered play generally follows the child’s lead within safe limits.
C. Both require the therapist to select every toy and direct every action.
D. Child-centered play prohibits reflection of feelings.

027. [CAC.909] A child throws a hard toy toward another person. Which limit-setting response is clearest and most supportive?

A. “Do whatever you want; all play is allowed.”
B. “You are bad and cannot play.”
C. “Tell me why you are aggressive before I stop you.”
D. “I won’t let you throw toys at people. You can throw this soft ball into the basket.”

CAC.910 | Cognitive Behavioral Therapy for Children

028. [CAC.910] A child notices, “I thought I would fail,” felt anxious, and avoided the assignment. Which CBT tool best introduces these links?

A. A logic model
B. The cognitive triangle linking thoughts, feelings, and behavior.
C. A genogram
D. A medication reconciliation form

029. [CAC.910] A child with low mood gradually schedules enjoyable, meaningful activities and reviews how mood and participation change. This is:

A. Behavioral activation
B. Aversive conditioning
C. A cross-sectional design
D. Family sculpting

030. [CAC.910] A counselor asks a child to examine evidence for a thought and develop a realistic alternative rather than insisting on positive thinking. This is:

A. Collaborative cognitive restructuring
B. Punishment
C. Unconditional exposure
D. A projective assessment

CAC.911 | Crisis Intervention Strategies

031. [CAC.911] At the start of an acute crisis contact, what should be prioritized?

A. Long-term insight before assessing danger.
B. Immediate safety, current danger, urgent needs, and available supports.
C. A full developmental history before any safety questions.
D. A diagnosis based on one statement.

032. [CAC.911] A youth and caregiver collaboratively identify warning signs, coping steps, supportive contacts, and emergency resources. This is:

A. A safety plan
B. A group rule sheet
C. A treatment termination summary
D. A norm-referenced assessment

033. [CAC.911] A counselor calls a receiving service with the family's permission, helps arrange the first contact, and confirms the next step. This is a:

A. Screening cutoff
B. Behavioral extinction
C. Cold referral only
D. Warm handoff

CAC.912 | Research Methods in Child Psychology

034. [CAC.912] A study defines "classroom participation" as the number of voluntary verbal contributions during a specified 20-minute lesson. This is an:

A. Research hypothesis only
B. External validity estimate
C. Operational definition
D. Random assignment procedure

035. [CAC.912] After participants are enrolled, a researcher uses a chance procedure to place them in one of two study conditions. This is:

A. Matched case selection
B. Random sampling from a population
C. Random assignment
D. Convenience recruitment

036. [CAC.912] A school-based study asks children to join after caregivers give required permission. The researcher explains the study in age-appropriate language and checks willingness. This is:

A. De-identified data analysis
B. Seeking child assent in research
C. Replacing all adult permission with a child's signature
D. A post hoc power calculation

CAC.913 | Neurodevelopmental Disorders

037. [CAC.913] A student has attention concerns reported at school. Before drawing diagnostic conclusions, what additional evidence is especially important?

A. Developmental history, impairment, and evidence across relevant settings and informants.
B. The student's grades without any contextual assessment.
C. An IQ score used as a diagnosis.
D. One classroom observation alone.

038. [CAC.913] A counselor recommends changing classroom materials and routines to improve access while respecting the student's communication and sensory profile. This best reflects:

A. A presumption that one accommodation fits everyone
B. Neurodiversity-affirming support
C. A deficit-only approach
D. A goal of eliminating all neurological differences

039. [CAC.913] A comprehensive evaluation of intellectual disability considers intellectual functioning and:

A. Only caregiver stress.
B. Only the student's attendance record.
C. A single symptom checklist without developmental history.
D. Adaptive functioning in conceptual, social, and practical life domains.

CAC.914 | Substance Abuse Counseling for Adolescents

040. [CAC.914] An adolescent substance-use screen is elevated. What is the best interpretation?

A. The adolescent definitely meets diagnostic criteria.
B. The score alone determines the required treatment.
C. The result should be ignored because screens never inform care.
D. The result indicates possible risk and need for further assessment; it is not itself a diagnosis.

041. [CAC.914] An adolescent is unsure about changing cannabis use. Which response best combines motivational interviewing with harm-reduction principles?

A. Promise that continued use is harmless.
B. End services unless the adolescent accepts the counselor's goal.
C. Explore the adolescent's own goals and ambivalence, and discuss practical risk-reduction steps without making abstinence a condition for care.
D. Confront the adolescent until they agree to abstain.

042. [CAC.914] Before asking an adolescent about substance use, the counselor should:

A. Explain confidentiality and its limits using current law and policy for that jurisdiction and service.
B. Promise absolute secrecy regardless of circumstances.
C. Avoid explaining privacy unless a disclosure occurs.
D. Assume adult and adolescent privacy rules are identical everywhere.

CAC.915 | School Counseling Practices

043. [CAC.915] A school organizes universal classroom lessons, targeted groups, and individualized referrals, then uses student data to guide priorities. This best describes:

A. A medication management service
B. An informal club without defined goals
C. A comprehensive, data-informed school counseling program
D. A single crisis response

044. [CAC.915] A counselor sets a measurable attendance goal and reviews attendance data every two weeks to decide whether to adjust supports. This is:

A. A one-time screening
B. Progress monitoring
C. A summative diagnosis
D. A random assignment procedure

045. [CAC.915] A teacher asks for help supporting a student, and the counselor collaborates on a classroom strategy while clarifying the counselor's role. This is:

A. A mandated group placement
B. Consultation
C. A direct diagnostic service to the whole class
D. Individual psychotherapy delivered to the teacher

CAC.916 | Parenting and Child Development

046. [CAC.916] Which parenting pattern is most associated with warmth, responsiveness, and clear age-appropriate expectations?

A. Uninvolved parenting
B. Authoritarian parenting
C. Authoritative parenting
D. Permissive parenting

047. [CAC.916] A distressed young child calms with a caregiver's steady voice and predictable presence before trying a coping skill independently. This illustrates:

A. Cognitive restructuring
B. A cohort effect
C. Co-regulation
D. Random reinforcement

048. [CAC.916] A caregiver gives specific praise immediately after a child asks for help appropriately, and that behavior becomes more frequent. The praise functions as:

A. Extinction
B. Positive reinforcement
C. Negative reinforcement
D. A discriminative diagnosis

CAC.917 | Psychopharmacology for Counselors

049. [CAC.917] A client reports a troublesome medication side effect and asks the counselor to change the dose. The counselor should:

A. Tell the client to stop all medication immediately.
B. Document the report and facilitate timely contact with the qualified prescriber rather than changing the dose.
C. Adjust the prescription without consulting the prescriber.
D. Recommend doubling the dose for one week.

050. [CAC.917] Pharmacokinetics primarily concerns:

A. How a counselor forms a therapeutic alliance.
B. How a child develops adaptive skills.
C. How the medication affects the body's systems.
D. How the body absorbs, distributes, metabolizes, and eliminates a medication.

051. [CAC.917] A counselor shares a client's observed sleep and mood changes with the prescriber after obtaining appropriate permission. This most directly demonstrates:

A. A diagnostic conclusion based on medication response
B. A waiver of informed consent
C. Independent prescribing
D. Interprofessional collaboration within role boundaries

CAC.918 | Special Topics in Adolescent Mental Health

052. [CAC.918] A teenager reports nonsuicidal self-injury. Which response is most appropriate?

A. Punish the behavior to prevent future disclosure.
B. Assume there is no suicide risk because the teen denies intent once.
C. Ask for graphic details in front of peers.
D. Ask compassionately about the behavior's function, injury, and suicide risk; develop safety supports and refer as indicated.

053. [CAC.918] A transgender adolescent tells a counselor their name and pronouns. Which response is most supportive?

A. Use the stated name and pronouns, explore the adolescent's goals, and assess support and safety without pathologizing identity.
B. Insist on using the name on an old record in every setting.
C. Delay support until the adolescent makes a permanent decision.
D. Treat gender identity itself as a disorder.

054. [CAC.918] A teen's late-night online activity is associated with reduced sleep and missed morning classes. A useful first step is to:

A. Assume all online activity is harmful.
B. Explore the teen's own digital habits, sleep pattern, goals, and feasible changes collaboratively.
C. Confiscate every device without assessment or discussion.
D. Diagnose an internet-use disorder from one report.

CAC.919 | Advocacy in Child and Adolescent Services

055. [CAC.919] A youth helps design the questions for a service review and later sees how their feedback influenced program changes. This is:

A. A unilateral administrative decision
B. Meaningful youth voice
C. Token participation
D. A norm-referenced test

056. [CAC.919] Several agencies document that transportation barriers prevent families from reaching services and jointly ask the county to fund accessible routes. This is:

A. A child assent procedure
B. Systems advocacy
C. Clinical supervision
D. Individual case diagnosis

057. [CAC.919] Before contacting a school on a young client's behalf, an advocate should first:

A. Discuss the plan, obtain appropriate permission, and consider confidentiality and possible consequences.
B. Send the full clinical record without permission.
C. Assume advocacy always overrides privacy.
D. Contact every organization connected to the family.

CAC.920 | Social Justice in Counseling

058. [CAC.920] A school offers the same online counseling option to everyone, but some families lack reliable devices or private internet access. An equity-oriented response is to:

A. Keep the identical option and assume equal access.
B. Identify the access barriers and provide feasible alternatives so students can participate.
C. Remove counseling from students without devices.
D. Blame families for not using the service.

059. [CAC.920] A counselor's case formulation includes housing insecurity and discriminatory school discipline as contributors to distress. This reflects:

A. A cultural stereotype
B. A claim that individual experience does not matter
C. A behavior-only explanation
D. Attention to structural conditions

060. [CAC.920] A practitioner pauses before making a referral and checks whether an unexamined assumption about a family's identity influenced the decision. This is an example of:

A. Applying a standardized norm
B. Using random assignment
C. Avoiding cultural discussion
D. Reflecting on implicit bias

CAC.921 | Lifespan Developmental Counseling

061. [CAC.921] A counselor recognizes that development may involve gains and losses across physical, cognitive, and relational domains and is shaped by context throughout life. This is a:

A. Single-stage model
B. Cross-sectional design
C. Fixed-trait explanation
D. Lifespan perspective

062. [CAC.921] Two age groups differ on a measure, but they also grew up during very different historical periods. This alternative explanation is a:

A. Treatment effect
B. Cohort effect
C. Extinction burst
D. Placebo response

063. [CAC.921] A family experiences multiple stressors, but stable relationships and community supports help a young person adapt over time. This best illustrates:

A. An absence of all risk
B. Developmental resilience supported by contextual resources
C. A guarantee that adversity has no effect
D. A fixed personality trait alone

CAC.922 | Behavioral Interventions for Children

064. [CAC.922] Before intervening for a recurring classroom behavior, a team records antecedents, the observable behavior, and what follows it. This is part of:

A. An attachment classification
B. Functional behavior assessment
C. A medication reconciliation
D. A cognitive interview only

065. [CAC.922] After a child appropriately requests a break, a difficult task is briefly paused; requests for breaks increase. The pause may be functioning as:

A. Positive reinforcement
B. Extinction
C. Negative reinforcement
D. Positive punishment

066. [CAC.922] A child learns to greet a counselor in one office but not in the classroom. To promote generalization, the team should:

A. Practice and reinforce the skill with varied people and settings.
B. Change the target skill without assessment.
C. Stop practice as soon as the child succeeds once.
D. Use the skill only in the original office.

CAC.923 | Advanced Research Methodology

067. [CAC.923] A study combines student outcome data with interviews and explicitly integrates the two types of findings to answer complementary questions. This is:

A. A cross-sectional survey only
B. A single-method experiment
C. Mixed-methods research
D. A case report with no integration

068. [CAC.923] Researchers ask whether an intervention's effect differs depending on school context. This is a question about:

A. Missing-data imputation
B. Mediation
C. Moderation
D. Internal consistency

069. [CAC.923] Before comparing questionnaire scores across language groups, a researcher tests whether the measure functions comparably. This concerns:

A. Measurement invariance
B. Treatment fidelity
C. A placebo response
D. Random assignment

CAC.960 | Dissertation

070. [CAC.960] A dissertation introduction identifies who is affected, what remains unknown, and why the gap matters. This section is primarily establishing the:

A. Research problem and knowledge gap
B. Study limitations only
C. Final statistical results
D. Reference list

071. [CAC.960] A student proposes an interview study but plans to answer a question about the causal effect of a treatment. The committee should first examine:

A. Whether the findings can be written before data collection.
B. Whether the title uses enough technical terms.
C. Alignment among the research question, purpose, design, measures, and analysis.
D. Whether limitations can be omitted.

072. [CAC.960] A dissertation reports that its sample was drawn from one urban clinic and cautions against assuming the findings transfer to every setting. This is an appropriate:

A. Informed assent form
B. Limitations statement
C. Preregistration record
D. Randomization schedule

CAC.924 | Ethical Issues in Child Welfare

073. [CAC.924] When evaluating a child-welfare plan, the team considers safe caregiving, continuity, and the child's relationships over time. The central planning aim is:

A. Immediate compliance by caregivers
B. A short-term placement without review
C. Permanency
D. Confidentiality alone

074. [CAC.924] A placement decision is being considered. Which ethical approach best respects the child's identity and relationships?

A. Share the child's confidential history with all community members.
B. Assume a single placement option fits every child.
C. Consider safe ways to preserve meaningful family, cultural, linguistic, and community connections.
D. Ignore cultural continuity because it is never relevant.

075. [CAC.924] When several interventions could protect a child, an ethical team should generally consider:

A. The least intrusive effective option consistent with safety and governing law.
B. The most restrictive option regardless of need.
C. The option that is easiest for the agency only.
D. No intervention until all uncertainty disappears.

CAC.925 | Mindfulness and Stress Reduction Techniques

076. [CAC.925] A counselor guides a client to notice thoughts as they arise and return attention to present sensations without judging the thoughts. This is:

A. Cognitive disputation
B. Mindfulness practice
C. A behavioral contingency
D. A diagnostic interview

077. [CAC.925] A child feels dizzy during a breathing exercise. The most appropriate response is to:

A. Treat discomfort as proof that the technique is working.
B. Continue without checking the child's experience.
C. Insist that the child take deeper breaths until the dizziness stops.
D. Pause or adapt the exercise and invite comfortable, unforced breathing or another grounding option.

078. [CAC.925] A young person chooses different coping tools for exam stress, family conflict, and sensory overload, then reviews which tools fit each situation. This reflects:

A. A single mandatory coping response
B. Coping flexibility
C. Measurement invariance
D. Extinction

CAC.926 | Technology in Counseling Practice

079. [CAC.926] At the start of a remote counseling session, what information is important for emergency planning?

A. The client's password for every device.
B. The counselor's favorite social media platform.
C. A promise that technology will never fail.
D. The client's current location and appropriate emergency contacts or procedures.

080. [CAC.926] A counselor wants to send sensitive information through a personal social-media account. The primary concern is:

A. Protecting digital confidentiality and using approved secure communication channels.
B. Replacing informed consent with an online message.
C. Improving the client's screen time.
D. Increasing the number of followers.

081. [CAC.926] A student cannot join video sessions because home internet is unreliable and the room is shared. A technology-equity response is to:

A. Label the student unmotivated.
B. Ask about access and privacy barriers and offer feasible alternatives.
C. Require the student to purchase new equipment.
D. Continue video-only sessions without discussion.

CAC.927 | Collaborative Practices with Schools and Communities

082. [CAC.927] A school counselor, clinician, and caregiver agree on shared goals, individual responsibilities, and a date to review progress. This creates a:

A. Clinical diagnosis
B. One-way referral letter
C. Unstructured information exchange
D. Shared care plan

083. [CAC.927] Before sending student information to a community provider, a school counselor should:

A. Send the entire school file automatically.
B. Assume collaboration removes privacy requirements.
C. Confirm appropriate authorization and share only information needed for coordinated support.
D. Share information with any partner who requests it.

084. [CAC.927] A neighborhood coalition maps youth clubs, trusted community leaders, existing services, and local skills before proposing a new program. This is:

A. A treatment randomization
B. A clinical diagnosis
C. A deficit-only needs assessment
D. Community asset mapping

CAC.928 | Advanced Techniques in Family Therapy

085. [CAC.928] A therapist invites family members to discuss a current disagreement directly in session while observing their interaction. This technique is:

A. Progress monitoring
B. A norm-referenced assessment
C. Behavioral activation
D. Enactment

086. [CAC.928] A therapist reframes a child’s repeated checking as a possible attempt to feel safe while also addressing its impact on family routines. This is:

A. Triangulation
B. Reframing
C. A screening diagnosis
D. Random assignment

087. [CAC.928] A family therapist proposes that a child’s withdrawal may help reduce caregiver conflict, then asks the family whether this tentative idea fits. This is a:

A. Fixed interpretation that cannot be revised
B. Systemic hypothesis
C. Definitive diagnosis from one observation
D. Medication reconciliation

CAC.929 | Clinical Supervision and Consultation

088. [CAC.929] A supervisor sets skill-development goals and reviews a supervisee’s recorded counseling practice. This is primarily:

A. A warm referral
B. Formative supervision
C. Restorative supervision only
D. Gatekeeping without feedback

089. [CAC.929] Which feedback is most useful to a supervisee?

A. “Be better next time.”
B. “Everything was fine; no need to review the session.”
C. “You are not a natural counselor.”
D. “You reflected the youth’s feeling before asking the next question; next time, allow more pause for their response.”

090. [CAC.929] A supervisor identifies a serious competence concern that may place clients at risk. The supervisor should:

A. Ignore the concern to preserve the relationship.
B. Ask the supervisee to decide alone whether clients are safe.
C. Document concerns, provide appropriate remediation, and take protective action as needed.
D. Keep assigning cases beyond the supervisee’s competence.

CAC.930 | Program Evaluation and Outcome Measurement

091. [CAC.930] A program map links staff and funding to activities, outputs, and intended student outcomes. This is a:

A. Genogram
B. Behavioral chain only
C. Research consent form
D. Logic model

092. [CAC.930] An evaluator examines whether participants were reached, sessions were delivered as planned, and implementation barriers occurred. This is primarily:

A. Outcome evaluation only
B. A randomized trial by definition
C. Process evaluation
D. A diagnosis

093. [CAC.930] A school tests a small scheduling change, reviews attendance data, and uses results to refine the next cycle. This is:

A. Continuous quality improvement
B. A placebo condition
C. A one-time summative report with no follow-up
D. A cohort effect

CAC.931 | Advanced Pedagogy

094. [CAC.931] A course outcome requires students to conduct a functional assessment, but the final exam only asks them to define vocabulary. The main design problem is weak:

A. Group cohesion
B. Random assignment
C. Cohort matching
D. Constructive alignment

095. [CAC.931] An instructor uses a low-stakes exit question during a unit and changes the next lesson based on student responses. This is:

A. Formative assessment
B. A dissertation defense
C. A clinical screening
D. Summative assessment only

096. [CAC.931] A course provides readings in accessible formats, offers varied ways to engage, and lets students demonstrate learning through more than one appropriate format. This best reflects:

A. A single fixed teaching method
B. Universal Design for Learning
C. Norm-referenced grading
D. A logic model

INTEGRATIVE | Cross-Course Questions

097. [INTEGRATIVE] A child's behavior is understood within reciprocal family patterns, while the counselor also considers developmental level and caregiver capacity. Which formulation best integrates these perspectives?

A. Describe the interactional cycle and developmental context, then collaborate with caregivers on an age-appropriate change.
B. Attribute the entire problem to the child and ignore family interaction.
C. Choose an intervention before gathering contextual information.
D. Assume the behavior is fixed because it appeared early.

098. [INTEGRATIVE] After a crisis, a youth is stabilized and the team needs to coordinate school and community follow-up. Which next step best combines crisis continuity and collaborative practice?

A. With appropriate permission, make a warm handoff, clarify each partner's role, and agree on follow-up.
B. End contact once the youth leaves the crisis setting.
C. Send the full clinical record to every partner without checking authorization.
D. Ask the school to manage all clinical risk without coordination.

099. [INTEGRATIVE] A researcher wants to compare counseling outcomes across language groups. Which step best combines sound measurement and culturally responsive research?

A. Translate the measure literally and treat translation as proof of equivalence.
B. Assume identical score meanings without testing them.
C. Remove participants whose results differ from expectations.
D. Examine whether the measure functions comparably across groups and interpret findings in cultural context.

100. [INTEGRATIVE] A school's data show that a service is underused by youth facing transportation and language barriers. Which response best integrates evaluation, advocacy, and youth voice?

A. Publish identifiable student records to attract attention.
B. Change the program without consulting affected youth.
C. Conclude that students do not need the service because attendance is low.
D. Review access data with youth and community partners, identify barriers, and test an accessible service change.

Answer Key

Correct answers and brief rationales | Questions 001–100

The answer key begins after all 100 questions. Question numbers match the numbered items in the first section.

CAC.901 | Advanced Developmental Psychology

001. D — Transactional model
Rationale: The transactional model emphasizes reciprocal influence between the child and the environment over time.

002. A — A developmental cascade
Rationale: A developmental cascade describes change in one domain influencing later outcomes in other domains.

003. A — Ecological systems perspective
Rationale: An ecological perspective considers nested settings and broader cultural, institutional, and historical conditions.

CAC.902 | Theories of Counseling and Psychotherapy

004. C — Reflect both sides of the ambivalence and invite the adolescent to describe their own reasons for change.
Rationale: Motivational interviewing explores ambivalence and evokes the client's own motivation while supporting autonomy.

005. D — Empathic understanding, congruence, and unconditional positive regard.
Rationale: These relational conditions are central person-centered concepts.

006. C — Solution-focused brief therapy
Rationale: Solution-focused work often explores exceptions and resources that support a preferred future.

CAC.903 | Child and Adolescent Assessment Techniques

007. C — Reliability
Rationale: Reliability concerns score consistency or precision under relevant conditions.

008. A — Use the result to guide follow-up assessment; do not treat the screening score alone as a diagnosis.
Rationale: Screening identifies possible need for further evaluation; diagnosis requires a fuller assessment under applicable standards.

009. B — Integrate the reports in context and investigate why perspectives differ.
Rationale: Informants observe different settings; discrepancies are data to understand rather than errors to erase.

CAC.904 | Ethics in Counseling Children and Adolescents

010. C — Assent: the child's informed, affirmative willingness to participate.
Rationale: Assent supports the child's meaningful participation and is distinct from an adult's legal consent.

011. B — Informed consent
Rationale: Informed consent includes relevant information and voluntary authorization before services or procedures.

012. C — Consult current jurisdictional requirements and follow applicable reporting procedures, explaining limits of privacy as appropriate.
Rationale: Reporting duties are jurisdiction-specific; counselors must follow applicable law and professional policy.

CAC.905 | Cultural Competence in Counseling

013. A — Cultural humility
Rationale: Cultural humility involves ongoing self-reflection, openness, and accountability about the limits of one's knowledge.

014. D — Intersectionality
Rationale: Intersectionality examines how interlocking identities and systems of power shape lived experience.

015. D — Cultural adaptation
Rationale: Thoughtful adaptation can improve cultural fit while preserving and monitoring the intervention's core mechanisms.

CAC.906 | Trauma-Informed Care Practices

016. A — Safety, choice, and collaboration
Rationale: Trauma-informed care prioritizes safety, choice, collaboration, trust, and empowerment.

017. A — Grounding
Rationale: Grounding supports present-moment orientation and can help stabilize attention during distress.

018. D — Support regulation and safety, then resume reflective work when the client can engage.
Rationale: The window-of-tolerance model suggests reflective work is more feasible when arousal is manageable.

CAC.907 | Family Systems Theory

019. C — Circular causality
Rationale: Circular causality focuses on reciprocal patterns in which each person's behavior influences and is influenced by others.

020. B — Triangulation
Rationale: Triangulation occurs when a third person is drawn into tension between two others.

021. C — Subsystem boundaries
Rationale: Boundaries regulate roles and contact between family members and subsystems.

CAC.908 | Group Counseling for Youth

022. A — Group cohesion
Rationale: Cohesion refers to members' sense of connection, trust, and commitment to the group.

023. A — A realistic explanation of confidentiality limits in a group.
Rationale: Group leaders should establish privacy expectations while being honest about limits outside their control.

024. A — Screen goals, readiness, safety needs, and fit with the group's purpose.
Rationale: Group-fit screening considers needs, readiness, safety, and compatibility with the group's goals.

CAC.909 | Play Therapy Techniques

025. B — "You stacked the blocks high, and now you knocked them down."
Rationale: Tracking describes observable play without adding an unsupported interpretation or taking over the play.

026. B — Directive play uses therapist-selected activities for goals; child-centered play generally follows the child's lead within safe limits.
Rationale: Directive work is therapist-guided; child-centered work emphasizes the child's lead and the therapeutic relationship.

027. D — "I won't let you throw toys at people. You can throw this soft ball into the basket."
Rationale: Effective limits protect safety, acknowledge the boundary, and offer an acceptable alternative.

CAC.910 | Cognitive Behavioral Therapy for Children

028. B — The cognitive triangle linking thoughts, feelings, and behavior.
Rationale: The cognitive triangle helps children see how thoughts, feelings, and actions relate.

029. A — Behavioral activation
Rationale: Behavioral activation increases engagement with valued or rewarding activities and monitors effects.

030. A — Collaborative cognitive restructuring
Rationale: Cognitive restructuring collaboratively evaluates thoughts and develops balanced, useful alternatives.

CAC.911 | Crisis Intervention Strategies

031. B — Immediate safety, current danger, urgent needs, and available supports.
Rationale: Crisis assessment first addresses immediate safety and urgent needs, then guides next steps.

032. A — A safety plan
Rationale: A safety plan is individualized and specifies practical steps if risk increases.

033. D — Warm handoff
Rationale: A warm handoff actively connects a client to the next service and supports continuity.

CAC.912 | Research Methods in Child Psychology

034. C — Operational definition
Rationale: An operational definition specifies how a construct is observed or measured.

035. C — Random assignment
Rationale: Random assignment allocates participants to conditions by chance; it is distinct from sampling.

036. B — Seeking child assent in research
Rationale: Child assent is an age-appropriate affirmative agreement sought in addition to required adult permission.

CAC.913 | Neurodevelopmental Disorders

037. A — Developmental history, impairment, and evidence across relevant settings and informants.
Rationale: Assessment should consider developmental history and functional impact across relevant contexts.

038. B — Neurodiversity-affirming support
Rationale: Neurodiversity-affirming practice emphasizes access, individual strengths, and meaningful supports rather than unnecessary normalization.

039. D — Adaptive functioning in conceptual, social, and practical life domains.
Rationale: Adaptive functioning is assessed alongside intellectual functioning and developmental history.

CAC.914 | Substance Abuse Counseling for Adolescents

040. D — The result indicates possible risk and need for further assessment; it is not itself a diagnosis.
Rationale: Screening is a first step that can guide assessment; it does not establish a diagnosis by itself.

041. C — Explore the adolescent's own goals and ambivalence, and discuss practical risk-reduction steps without making abstinence a condition for care.
Rationale: Motivational interviewing supports autonomy, while harm reduction seeks practical risk reduction without withholding care.

042. A — Explain confidentiality and its limits using current law and policy for that jurisdiction and service.
Rationale: Privacy requirements depend on jurisdiction and service context and should be explained accurately in advance.

CAC.915 | School Counseling Practices

043. C — A comprehensive, data-informed school counseling program
Rationale: Comprehensive programs coordinate planned services across student needs and use data to guide improvement.

044. B — Progress monitoring
Rationale: Repeated measurement of a target outcome helps assess response and guide adjustments.

045. B — Consultation
Rationale: Consultation is collaborative and indirect, supporting a consultee to help a student.

CAC.916 | Parenting and Child Development

046. C — Authoritative parenting
Rationale: Authoritative parenting combines responsiveness with consistent, developmentally appropriate limits.

047. C — Co-regulation
Rationale: Co-regulation is adult support that helps a child regulate before independent regulation is established.

048. B — Positive reinforcement
Rationale: A consequence added after a behavior that increases that behavior is positive reinforcement.

CAC.917 | Psychopharmacology for Counselors

049. B — Document the report and facilitate timely contact with the qualified prescriber rather than changing the dose.
Rationale: Counselors must stay within scope and coordinate medication concerns with the prescriber.

050. D — How the body absorbs, distributes, metabolizes, and eliminates a medication.
Rationale: Pharmacokinetics describes the body's handling of a medication; pharmacodynamics describes medication effects.

051. D — Interprofessional collaboration within role boundaries
Rationale: Coordinated communication can support safe care while respecting professional scope and permission.

CAC.918 | Special Topics in Adolescent Mental Health

052. D — Ask compassionately about the behavior's function, injury, and suicide risk; develop safety supports and refer as indicated.
Rationale: Nonsuicidal self-injury and suicide risk require careful, direct, compassionate assessment; one does not rule out the other.

053. A — Use the stated name and pronouns, explore the adolescent's goals, and assess support and safety without pathologizing identity.
Rationale: Respectful, affirming support honors the youth's expressed identity and attends to context and safety.

054. B — Explore the teen's own digital habits, sleep pattern, goals, and feasible changes collaboratively.
Rationale: Digital wellbeing is assessed in relation to the individual's functioning, sleep, relationships, and goals.

CAC.919 | Advocacy in Child and Adolescent Services

055. B — Meaningful youth voice
Rationale: Meaningful youth voice gives young people real influence over decisions affecting services.

056. B — Systems advocacy
Rationale: Systems advocacy seeks policy or resource changes affecting groups of people.

057. A — Discuss the plan, obtain appropriate permission, and consider confidentiality and possible consequences.
Rationale: Ethical advocacy balances client involvement, consent, privacy, and foreseeable impacts.

CAC.920 | Social Justice in Counseling

058. B — Identify the access barriers and provide feasible alternatives so students can participate.
Rationale: Equity attends to differing barriers and provides supports that enable meaningful access.

059. D — Attention to structural conditions
Rationale: Social justice counseling considers structural conditions alongside individual experiences and strengths.

060. D — Reflecting on implicit bias
Rationale: Reflecting on implicit bias can help identify automatic assumptions that affect professional judgment.

CAC.921 | Lifespan Developmental Counseling

061. D — Lifespan perspective
Rationale: The lifespan perspective views development as lifelong, multidimensional, multidirectional, and contextual.

062. B — Cohort effect
Rationale: Cohort effects arise from shared historical experiences and can complicate age comparisons.

063. B — Developmental resilience supported by contextual resources
Rationale: Resilience is a dynamic process supported by personal, relational, and contextual resources.

CAC.922 | Behavioral Interventions for Children

064. B — Functional behavior assessment
Rationale: Functional assessment gathers contextual data about antecedents, behavior, consequences, and likely function.

065. C — Negative reinforcement
Rationale: Removing an aversive condition after behavior can increase that behavior, which is negative reinforcement.

066. A — Practice and reinforce the skill with varied people and settings.
Rationale: Generalization involves transfer of a learned skill to people, settings, or materials beyond training conditions.

CAC.923 | Advanced Research Methodology

067. C — Mixed-methods research
Rationale: Mixed-methods designs intentionally integrate qualitative and quantitative evidence.

068. C — Moderation
Rationale: Moderation asks whether the strength or direction of a relationship varies with another variable.

069. A — Measurement invariance
Rationale: Measurement invariance supports the interpretation that a measure operates comparably across groups.

CAC.960 | Dissertation

070. A — Research problem and knowledge gap
Rationale: A focused problem statement explains significance and the gap the study will address.

071. C — Alignment among the research question, purpose, design, measures, and analysis.
Rationale: Methodological alignment ensures the proposed design can answer the stated research questions.

072. B — Limitations statement
Rationale: A limitations statement explains constraints on interpretation or transferability.

CAC.924 | Ethical Issues in Child Welfare

073. C — Permanency
Rationale: Permanency planning seeks a safe, stable, enduring family arrangement for the child.

074. C — Consider safe ways to preserve meaningful family, cultural, linguistic, and community connections.
Rationale: Cultural continuity and safety should be considered together in child-welfare planning.

075. A — The least intrusive effective option consistent with safety and governing law.
Rationale: A least-intrusive approach seeks effective protection with no more restriction than necessary.

CAC.925 | Mindfulness and Stress Reduction Techniques

076. B — Mindfulness practice
Rationale: Mindfulness involves purposeful present-moment attention with openness and reduced automatic judgment.

077. D — Pause or adapt the exercise and invite comfortable, unforced breathing or another grounding option.
Rationale: Breathing exercises should be comfortable and adjusted or stopped if they cause distress or discomfort.

078. B — Coping flexibility
Rationale: Coping flexibility means selecting and adapting strategies to fit changing demands and contexts.

CAC.926 | Technology in Counseling Practice

079. D — The client's current location and appropriate emergency contacts or procedures.
Rationale: Telehealth contingency planning includes current location and steps for responding to urgent events or disconnection.

080. A — Protecting digital confidentiality and using approved secure communication channels.
Rationale: Digital confidentiality requires secure, approved systems and careful handling of sensitive information.

081. B — Ask about access and privacy barriers and offer feasible alternatives.
Rationale: Equitable telehealth practice accounts for devices, bandwidth, private space, and digital skills.

CAC.927 | Collaborative Practices with Schools and Communities

082. D — Shared care plan
Rationale: A shared care plan clarifies goals, roles, and follow-up across partners.

083. C — Confirm appropriate authorization and share only information needed for coordinated support.
Rationale: Information sharing should be permission-based, limited, and consistent with applicable privacy requirements.

084. D — Community asset mapping
Rationale: Asset mapping identifies existing strengths, relationships, and resources that can support community goals.

CAC.928 | Advanced Techniques in Family Therapy

085. D — Enactment
Rationale: Enactment lets the therapist observe live interaction patterns in the session.

086. B — Reframing
Rationale: Reframing offers an alternative meaning that can reduce blame while still addressing consequences.

087. B — Systemic hypothesis
Rationale: A systemic hypothesis is provisional and should be tested and revised collaboratively.

CAC.929 | Clinical Supervision and Consultation

088. B — Formative supervision
Rationale: Formative supervision develops knowledge, skills, and clinical judgment.

089. D — “You reflected the youth’s feeling before asking the next question; next time, allow more pause for their response.”
Rationale: Specific feedback identifies observable behavior, its effect, and an actionable next step.

090. C — Document concerns, provide appropriate remediation, and take protective action as needed.
Rationale: Gatekeeping includes addressing competence concerns and protecting clients and the profession.

CAC.930 | Program Evaluation and Outcome Measurement

091. D — Logic model
Rationale: A logic model explains how resources and activities are expected to lead to outcomes.

092. C — Process evaluation
Rationale: Process evaluation considers implementation, reach, dose, fidelity, and context.

093. A — Continuous quality improvement
Rationale: Continuous quality improvement iteratively uses data to test and refine service delivery.

CAC.931 | Advanced Pedagogy

094. D — Constructive alignment
Rationale: Constructive alignment connects intended outcomes, learning activities, and assessments.

095. A — Formative assessment
Rationale: Formative assessment gathers evidence during learning to guide immediate instructional decisions.

096. B — Universal Design for Learning
Rationale: UDL plans flexible engagement, representation, and means of action or expression.

INTEGRATIVE | Cross-Course Integrative Questions

097. A — Describe the interactional cycle and developmental context, then collaborate with caregivers on an age-appropriate change.
Rationale: Developmental and family-systems perspectives both emphasize context, change over time, and reciprocal relationships.

098. A — With appropriate permission, make a warm handoff, clarify each partner’s role, and agree on follow-up.
Rationale: A warm handoff and clear shared roles support continuity while respecting permission and privacy.

099. D — Examine whether the measure functions comparably across groups and interpret findings in cultural context.
Rationale: Measurement invariance and cultural context are important before comparing groups.

100. D — Review access data with youth and community partners, identify barriers, and test an accessible service change.
Rationale: Data-informed improvement is stronger when affected youth participate and structural access barriers are addressed.